TE-09 Chemical / HazMat Exposure

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | Reconciled October 30, 2026

Responder safety and contamination control come before patient contact. Do not enter a hot zone or confined space without required training, monitoring, PPE, respiratory protection, and HazMat coordination.

Universal chemical-exposure flow

Step	Approved action
1. Zones and PPE	Establish hot, warm, and cold zones; identify wind, terrain, vapor density, flammability, oxygen-deficiency, and confined-space risk; request HazMat and incident command.
2. Safe rescue	Remove from the source only by properly trained and protected personnel. Prevent additional victims and protect EMS equipment from contamination.
3. Decontaminate	Remove clothing/jewelry; brush dry agents; blot liquids; irrigate when indicated; contain runoff; and do not bring contamination into the ambulance or receiving facility.
4. ABCs	Treat airway edema, bronchospasm, respiratory failure, shock, dysrhythmia, seizure, burns, and temperature injury under applicable approved pathways.
5. Identify agent	Use label, placard, SDS, ERG, witness report, route, duration, concentration, and symptoms without re-entering the hazard or transporting contaminated packaging.
6. Consult and transport	Call Poison Help (1-800-222-1222) and Medical Control for significant or unknown exposure. Verify destination and notify the receiving facility of decontamination status.

Provider-level actions

Level	Approved actions
EMT	Stage until safe for the assigned PPE/training level; assist clothing removal and irrigation; support ABCs; obtain product/exposure details safely; and contact Poison Help.
AEMT	All EMT care plus IV/IO access, authorized fluids/medications, continuous reassessment, and preparation for respiratory deterioration. For cholinergic findings, use TE-08 for supportive care and escalation; no CHEMPACK/Mark I field kit is carried.
Paramedic	All prior care plus advanced airway/ventilation, CPAP when appropriate, cardiac monitoring, 12-lead, and seizure, dysrhythmia, and shock management. Specialty antidotes require an approved kit/order or Medical Control.

Universal safeguards

- Do not routinely neutralize acids, alkalis, or other chemicals on or inside the patient. Do not induce vomiting after caustic, hydrocarbon, or unknown ingestion.

- A normal initial examination or pulse-ox reading does not exclude delayed pulmonary edema, cellular asphyxiation, methemoglobinemia, electrolyte toxicity, or deep tissue injury.

Chlorine / chloramine and ammonia

Agent	Recognition and field care
Chlorine / chloramine	Pool products, industrial chlorine, or mixing bleach with acid/ammonia can cause eye/throat burning, cough, wheeze, stridor, hypoxemia, delayed pulmonary edema, and frostbite from liquefied product. Protected removal, clothing removal, irrigation, ventilation, and bronchospasm care are priorities. Nebulized bicarbonate is not routine and requires separate adoption/stock/order.
Ammonia	Fertilizer, refrigeration, or concentrated cleaners can cause severe eye and upper-airway injury, laryngeal edema, chemical burns, pulmonary edema, and frostbite. Use protected rescue, decontamination, irrigation, airway vigilance, and early difficult-airway preparation.

Organophosphate / carbamate pesticide

Topic	Required action
Recognition	Consider after pesticide exposure with miosis, sweating, salivation, lacrimation, vomiting, diarrhea, urination, fasciculations, weakness, bradycardia, bronchospasm, or profuse pulmonary secretions.
Field priorities	Avoid secondary dermal contamination. Use chemical PPE, remove/bag clothing, wash skin/hair, suction secretions, oxygenate, ventilate, and use TE-08 for cholinergic recognition and escalation.
Antidote boundary	Claiborne ground units do not carry a CHEMPACK, Mark I, or equivalent atropine/pralidoxime kit. Do not use legacy kit counts, autoinjector directions, or unstocked antidote doses.

Hydrogen sulfide

Topic	Required action
Risk	Sewers, septic systems, manure pits, petroleum operations, and decaying organic matter may generate a heavier-than-air gas that rapidly incapacitates rescuers. Odor is unreliable because olfactory fatigue occurs.
Field care	Only trained personnel with appropriate respiratory protection may remove the patient. Provide high-concentration oxygen and ventilation, cardiac monitoring, seizure care, and Poison Help/Medical Control consultation. Nitrite therapy requires separate adoption/stock/order.

Hydrofluoric acid / hydrogen fluoride

Topic	Required action
Recognition	Glass etching, rust removal, industrial processing, wheel/metal cleaners, and laboratory products can cause delayed pain and systemic hypocalcemia, hypomagnesemia, hyperkalemia, prolonged QT, ventricular dysrhythmia, shock, or death.
Field care	Use chemical PPE; remove clothing/jewelry; brush dry material; irrigate immediately and copiously; establish access in an unexposed extremity when feasible; obtain continuous ECG monitoring; and obtain Poison Help/Medical Control guidance.
Calcium boundary	After irrigation, topical, nebulized, or other calcium gluconate routes require a finalized Claiborne kit/preparation, training, standing order, or direct Medical Control. Eye, inhalation, ingestion, hand, face, genital, circumferential, large-area, or symptomatic exposure requires urgent specialty evaluation.

Unknown chemical / corrosive

Topic	Required action
Approach	Treat unknown material as hazardous. Use placards, labels, SDS, ERG, witnesses, and toxidrome without touching contamination or bringing a container into the ambulance.
Decontamination	Brush dry agents before irrigation. For most liquid skin/eye exposures, remove contaminated items and irrigate with copious tepid water. Do not apply water to a suspected water-reactive material until HazMat/Poison Help provides direction.
Disposition	Transport symptomatic patients and those with significant eye injury, airway symptoms, abnormal vital signs, altered mental status, intentional exposure, concentrated corrosive ingestion, uncertain decontamination, or delayed-risk agents.

Responder exposure

- Any responder with symptoms, PPE breach, splash, inhalation, needlestick, or uncertain exposure must be removed from duty, decontaminated, medically evaluated, and documented under the agency exposure process.

- Use an unexposed extremity for vascular access when feasible and protect patient-care equipment from contamination.

Poison Help, Medical Control, and destination

Topic	Required action
Poison Help	Call 1-800-222-1222 for severe/unknown exposure, unusual antidote, persistent symptoms, ingestion, intentional exposure, pediatric patient, pregnancy, or destination guidance.
Medical Control	Use for care outside standing orders, specialty antidotes, unapproved preparations, unusual agent-specific treatment, or destination decisions requiring local resources.
Destination	Coordinate early with the most appropriate receiving facility, burn center, trauma center, pediatric center, or toxicology-capable facility. Notify before arrival and report decontamination status.
Handoff	Report identified/suspected agent, source, route, concentration, duration, PPE/zone, decontamination, product information, Poison Help/Medical Control recommendations, treatment, response, and contamination status.

Specialty-therapy boundary

Topic	Required action
Not routine	Nebulized sodium bicarbonate, calcium gluconate for hydrofluoric acid, methylene blue, nitrite therapy, chelation, and other specialty antidotes are not routine field therapies.
Required before use	A specialty therapy may appear in field care only when specifically adopted, stocked, trained, and ordered under a finalized Claiborne standing order or direct Medical Control.
Nerve-agent kits	Claiborne ground units do not carry a CHEMPACK, Mark I, or equivalent atropine/pralidoxime field kit. Use TE-08 for scene safety, decontamination, recognition, and escalation rather than legacy kit directions.

Release note

- Current Tennessee law, provider scope, Poison Help recommendations, HazMat direction, and Claiborne County policy control if any conflict exists.

- This direct source PDF matches Claiborne-approved TE-09 app content reconciled October 30, 2026.

Critical reminder: Do not become an additional victim. Safe rescue, contamination control, decontamination, airway/ventilation support, Poison Help, Medical Control, and early destination notification are the approved Claiborne field priorities.